

Sarah's Surgery Guide

Cervical disc replacement · surgery day Thursday, October 8, 2026 · Dr. Robert A. Rovner,
Disc & Spine of California, (925) 275-0700.

Inside: your details and care team, the day-by-day pre-op checklist (Oct 2–8), questions for your pre-op visit, the recovery calendar with real dates, printable daily logs, the red-flag card, Wesley's caregiver guide, and the surgery, recovery and risk pages. Compiled October 2, 2026.

Medical disclaimer: This private guide is general education compiled from published sources and the public website of Dr. Rovner's practice. It is not medical advice and does not replace the instructions Sarah gets from her surgical team. If anything here differs from what they say, follow them. For trouble breathing or rapidly increasing neck swelling, call 911 (see red-flag card).

Animations on the website appear here as still images. Numbers in brackets refer to the reference list at the end. Blank lines marked "To fill in" are for details not yet known. The deeper research pages are on the website only.

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Sarah's details and care team

Start here

Recovery updates for family

Dated posts and a "Day X" indicator, for your mom and family to check on their phones.

Meals & visits sign-up

Family can pick a meal, ride or short visit from Oct 8 to Oct 28. Sign-ups go to Wesley by text.

Pre-op checklist

A short list for each day from Oct 2–8.

What to know

The essentials on one page, without the research detail.



Recovery calendar

Real dates: week 1 is Oct 8–14, about 6 weeks is Thu, Nov 19, 3 months is Jan 8, 2027.



Daily recovery log

Printable pages for pain, medicines, walking, swallowing and the incision check.



When to call for help

When to call 911, when to call the surgeon, and what's usually normal.



Questions for your pre-op visit

What to ask Dr. Rovner's team before Thursday.



For Wesley: how to help

The first 72 hours, the first two weeks, meals, driving and what to watch for.

My surgery details

Surgery date	Thursday, October 8, 2026
Arrival time	To fill in: arrival time (the office or facility will tell you) _____
Level(s)	To fill in: C5–C6, C6–C7, or both _____
Implant	To fill in: implant name (ask the surgeon) _____
Surgeon	Robert A. Rovner, MD, MBA (orthopedic spine surgeon)
Hospital / surgery center	To fill in: where the surgery will be _____ To fill in: address _____
Surgeon's phone	(925) 275-0700 (main office line)
After-hours number	To fill in: after-hours / on-call number (not published on the practice site) _____
First post-op visit	To fill in: date and time of first follow-up _____
Notes	To fill in: anything else from the office _____

Your care team

Surgeon	Robert A. Rovner, MD , orthopedic spine surgeon. The practice lists him as "Robert A. Rovner, M.D., MBA" Stated by practice [74, 79]
Practice	Disc & Spine of California (with Vikram Talwar, MD) Stated by practice [73, 74]
Main office	1320 El Capitan Drive, Suite 200, Danville, CA 94526 Stated by practice [75]
Phone	(925) 275-0700 · Fax (925) 275-0701 Stated by practice [75, 84]
Email	appointments@discandspine.com (scheduling) Stated by practice [75]
Physician assistant	Joshua Ninichuck, PA-C, works with Dr. Rovner Stated by practice [82]
After hours	To fill in: after-hours number _____ To fill in No after-hours number is published on the website. Ask the office. For emergencies call 911.
Website	discandspine.com

A gentle word before Thursday

Feeling nervous is completely normal. One spine practice's patient manual says exactly that, and suggests that anyone treated for anxiety or depression check in with that clinician before surgery ^[92]. Disc replacement is done through a small incision at the front of the neck, usually in a skin crease, and takes about 2 to 3 hours ^[25]. Dr. Rovner's practice says many of its patients go home the same day ^[73].

The research behind this guide is still here under **Deeper reading** in the menu, if you ever want it. You don't need to read it to be ready.

Your Care Team: Dr. Rovner and Disc & Spine

Here is everything Dr. Rovner's practice publishes that's relevant to you, each item labeled with where it came from. If something isn't published, like your arrival time, your level, or an after-hours number, it's left as a placeholder for you to fill in.

Labels: **Stated by practice** the practice says this on its own website · **Public registry** public government registry · **Directory listing** commercial doctor directory, not verified · **Inferred** my inference, so please confirm · **To fill in** still needs to be filled in.

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About Dr. Rovner

- MD from the University of Maryland (1979); orthopedic surgery residency at UC Irvine; spine surgery fellowship at USC–Rancho Los Amigos (1984–85) **Stated by practice** [74].
- Board certified in orthopedic surgery since 1987 and recertified every 10 years; in private practice in the East Bay since 1984; clinical faculty at UC Davis School of Medicine **Stated by practice** [74].

- In practice with Dr. Vikram Talwar since 2005; together they founded Disc & Spine in 2015 and assist each other in surgery **Stated by practice** ^[74].
- The federal NPI registry lists him as an orthopaedic surgeon (spine) at the Danville address, phone (925) 275-0700 **Public registry** ^[84].
- The practice site links to the American Board of Orthopaedic Surgery's public certification-lookup tool. You can confirm board status there yourself **Stated by practice** ^[73].

Where the surgery happens

To fill in: hospital or surgery center _____ **To fill in**. The practice website does not say where its surgeries are done. A commercial directory lists Dr. Rovner as affiliated with John Muir Medical Center Walnut Creek and San Ramon Regional Medical Center **Directory listing** ^[85]. That does *not* tell you where your surgery will be. Confirm the location and arrival time with the office.

The practice says cervical disc problems are usually treated from the front of the neck, "either an outpatient procedure or done with a one-night stay for observation" **Stated by practice** ^[78].

A 2018 news post lists a second office for Dr. Rovner at 2923 Webster Street, #202, Oakland, CA 94609. Check that it is still current **Stated by practice** ^[81].

What the practice says about cervical disc replacement

- Surgery is done "through the front part of the neck," between the muscle and the swallowing and breathing tubes **Stated by practice** ^[77].
- Swallowing: the esophagus is gently moved aside, which "can cause a week or 2 of difficulty swallowing," and only rarely lasting problems **Stated by practice** ^[77].
- Voice: the nerve to the vocal cord can be irritated, "usually transient" **Stated by practice** ^[77].
- Infection, nerve injury and bleeding are described as very rare **Stated by practice** ^[77].
- Good candidates have a damaged disc pressing on a nerve root or the spinal cord, with only mild arthritis in the side (uncovertebral) and back (facet) joints **Stated by practice** ^[77].

Which implant?

Your implant: To fill in: implant name _____ **To fill in**.

The practice's disc replacement page, which is undated and doesn't name its author, says the writer had been implanting the Orthofix M6 disc for about 2 years, and it links an M6-C fact sheet from the manufacturer **Stated by practice** ^[77, 83]. That page appears to be older than later safety news. In January 2026 the UK regulator (MHRA) reported that the manufacturer discontinued the M6-C in February 2025, and it advises annual X-rays for people who already have one because of a risk of bone loss around the implant (osteolysis) ^[20]. So the website doesn't tell us which implant you'll get. Please ask the surgeon directly. It's one of the questions on your question list. **Inferred**

The practice's published post-op instructions

The practice publishes general post-op instructions plus short notes for specific operations, including disc replacement **Stated by practice** ^[76]. The guidance they give for disc replacement is to follow the wound-care rules, and "since there is no bone growth expectation, resume activities as tolerated." In summary:

Disc & Spine post-op instructions (paraphrased; quotes are the practice's words)	
Dressing	Remove it on day 2 or 3; if the wound is dry, leave it off. Some drainage is normal: use a dry gauze dressing and as little tape as possible. Antibiotic ointment isn't needed; dry blood can be gently wiped with peroxide and gauze.
Showering	Once the wound is sealed (3 days) and dry, shower normally and blot dry. Try not to remove the steri-strips. No baths or pools until the post-op visit.
Diet	Normally no restrictions. "This is not a good time to diet." Red meat and iron supplements help with post-op anemia.
Activity	Bed rest is discouraged; "go as far as you can" walking. Avoid "excessive" lifting, bending, twisting, sitting or standing. Running probably not early on.
Driving	"Not restricted beyond common sense and good judgement."
Fever	The page reads "Fever is normal in the first 35 days (even as high as 102+)," then says: if a fever over 101 persists, call the office during the day. "35 days" is very likely a typo for "3-5 days" Inferred . Please ask the office what fever should make you call.
Medicines	Usually a narcotic pain medicine (Tylenol may be substituted if it works), sometimes a muscle relaxant. Narcotics cause constipation, so use the minimum. Avoid NSAIDs (Advil, Aleve, etc.). No routine antibiotics after surgery.
Physical therapy	Generally not needed unless there's a specific reason.
Flying	Once the wound is healed and no visit is needed for a while.

Their separate note for *fusion* (a brace is for comfort only; avoid NSAIDs for 3 months) is written for fusion patients, not disc replacement ^[76]. Ask which notes apply to you.

What the website doesn't publish

- Pre-op instructions: fasting, which medicines to stop, arrival time. This guide uses general, reputable instructions instead, and labels them "confirm with your surgical team."
- An after-hours or on-call number.
- Where surgery is done, and which implants are currently used.
- Patient forms for Dr. Rovner's patients are on the [forms page](#) (registration packet and follow-up questionnaire) **Stated by practice** ^[80].

Where the practice's pages differ from other sources

- **Driving:** Dr. Rovner's page says common sense. Other sources say not while taking narcotic pain medicine, not before you can turn to check your blind spot ^[91, 93], or no driving for 2 weeks after spine surgery ^[88]. For you, ask the surgeon and don't drive on narcotics.
- **Fever:** the practice says call if a fever over 101 persists ^[76]. MedlinePlus says contact the surgeon for chills or a fever of 101°F or higher ^[88]; a Canadian hospital booklet uses 100.4°F ^[91].
- **Nerve maps:** the practice's page says C7 numbness travels to the ring and little fingers ^[77]. Merck Manual describes C7 pain going to the middle finger ^[30]. Real symptom maps overlap a lot ^[32].

Pre-op Countdown Checklist

A few small things each day, so nothing piles up. These are typical instructions from reputable sources. Your surgical team's instructions come first, so **confirm anything about medicines, fasting and timing with them**. Ticks are saved in this browser only.

Friday, October 2, 2026

Friday: calls and confirmations

- ☐ Call Dr. Rovner's office at (925) 275-0700. Confirm where the surgery is, the arrival time, the fasting rules, and the after-hours number. Then fill in My surgery details.
- ☐ Ask exactly **which medicines, vitamins and supplements to stop and when**. Stop them only as directed ^[25, 29]. Blood thinners and NSAIDs such as aspirin, ibuprofen and naproxen are often stopped 1–2 weeks before surgery, so ask today if no one has told you ^[86].
- ☐ Mention any diabetes or weight-loss medicines such as semaglutide or tirzepatide. Some need to be paused before surgery ^[86].
- ☐ Arrange the ride: someone must drive you home ^[29, 87]. Arrange a helper to stay with you, ideally overnight at first ^[92].
- ☐ If you smoke or use nicotine, stop. Ask the team for help ^[29, 25].
- ☐ Fill out Dr. Rovner's patient forms if the office asks you to ^[80]. Check insurance approval with your insurer ^[92].

Saturday, October 3, 2026

Saturday: get the house ready

- ☐ Set up a recliner or a pile of pillows. After neck surgery, many people sleep best propped upright ^[92].
- ☐ Move everyday things between shoulder and waist height: dishes, snacks, toiletries. Use counter-height appliances rather than low ovens ^[91].
- ☐ Clear tripping hazards such as small rugs and cords, add night-lights, and keep phones and emergency numbers within reach ^[91].
- ☐ Set up a phone or tablet stand at eye level by your resting spot, so you aren't looking down. The cervical spine booklet asks you to limit bending your neck up and down ^[91]. (The stand is my practical suggestion.)

Sunday, October 4, 2026

Sunday: food

- ☐ Cook and freeze single-serving meals for 2–4 weeks, or stock easy frozen meals ^[91].
- ☐ Stock soft, moist foods for a sore throat, such as scrambled eggs, soups, mashed potatoes, oatmeal, pudding and yogurt, plus popsicles and ice chips ^[90, 93]. See meal ideas.
- ☐ Ask the team about a stool softener. Constipation from pain medicine is common ^[76, 93].

Monday, October 5, 2026

Monday: bathroom and clothes

- ☐ Shower setup: a handheld showerhead if you have one, a non-slip mat, and toiletries left on the counter ^[91].
- ☐ Lay out loose, button-front or zip-front tops, so nothing pulls over your head (my practical suggestion). Plan comfortable going-home clothes that are easy to put on ^[86].
- ☐ Plan to fill pain prescriptions on the way home, so they're ready ^[88].

Tuesday, October 6, 2026

Tuesday: pack the bag

- ☐ ID, insurance card, and a list of all your medicines with doses, including the ones you stopped. Bring the bottles if possible ^[86, 87].
- ☐ Glasses (not contacts), toothbrush, phone and charger, a lightweight robe, flat non-slip shoes, and loose clothes ^[86, 91].
- ☐ CPAP if you use one, and any collar or brace the office gave you ^[87, 92].
- ☐ Leave jewelry and valuables at home ^[87].

Wednesday, October 7, 2026

Wednesday: the night before

- ☐ If you haven't heard your arrival time by afternoon, call the office. Many facilities call the day before; that is how one practice describes it ^[92].
- ☐ Call the surgeon's office if you have a fever, cough, cold or flu symptoms, chest pain, or a new rash ^[86].
- ☐ Shower with the soap they gave you, or antibacterial soap. Don't shave the front of your neck, and remove nail polish and makeup ^[86].
- ☐ Stop eating and drinking at the time you were told ^[86, 25].

Thursday, October 8, 2026

Thursday: surgery day

- ☐ Take only the medicines you were told to, with a small sip of water ^[87].
- ☐ Shower. No deodorant, lotion, perfume or makeup. Wear loose clothes and flat shoes, with no jewelry or contacts ^[87].
- ☐ Arrive at To fill in: arrival time _____. You may need to arrive up to 2 hours before surgery ^[87].
- ☐ A responsible adult drives you home. No driving for at least 24 hours ^[87].

Questions for Your Pre-op Appointment

Bring this to your pre-op appointment or call. Write the answers right on the printout. You don't need to ask everything. Star the ones that matter most to you.

About your operation

- ☐ Which level or levels: C5–C6, C6–C7, or both? Why disc replacement rather than fusion for me?
- ☐ Which implant will you use, and why that one? The practice site mentions the M6 disc, which has since been discontinued (see Which implant?) ^[77, 20].
- ☐ Is it possible you'll decide on fusion during surgery instead? If so, when would I find out?
- ☐ Where will it be done, and will I go home the same day or stay one night ^[78]?
- ☐ How long will surgery take? Who will update Wesley, and how?

Before surgery

- ☐ Which medicines and supplements do I stop, and on what date? When do I restart them ^[86]?
- ☐ When exactly do I stop eating and drinking? Are clear liquids allowed, and until when ^[87]?
- ☐ What time do I arrive, and who calls me with that time?
- ☐ Do I need any pre-op tests or medical clearance?

After surgery

- ☐ What is the after-hours number if something worries me at night?
- ☐ Which fever should make me call? (Your website says "35 days," which I think means 3–5 days ^[76].)
- ☐ Will I need a collar? When can I shower, and when should the dressing come off ^[76]?
- ☐ What will I take for pain? Can I use Tylenol, and what's the maximum? No NSAIDs, and for how long ^[76]?
- ☐ What are my limits for lifting, bending, and looking down at a phone or laptop?
- ☐ When can I drive? When can I work, and from home sooner ^[76, 25]?
- ☐ When is my first follow-up, and will there be X-rays?
- ☐ Will I get an implant card, and do I need any long-term imaging for this implant ^[20]?

For a longer list, see the full question list (deeper reading).

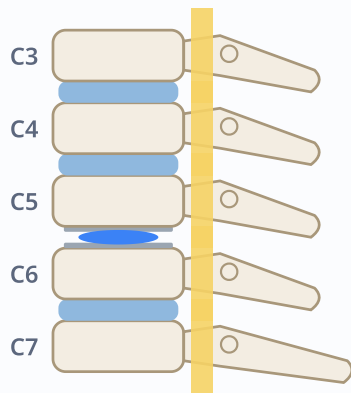
What to Know: The Short Version

The short version. Each point links to its source if you want to check it.

What the surgery is

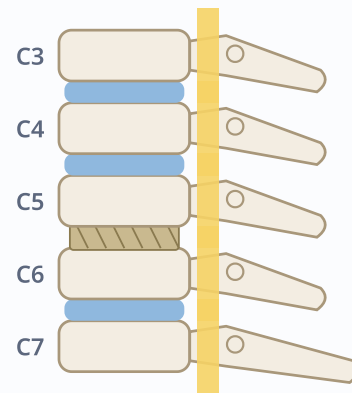
- The damaged disc pressing on a nerve is removed and replaced with a small moving implant, working from the front of the neck ^[25, 27].
- Unlike fusion, the treated level keeps moving. In one 10-year trial, the treated level still had about 8.7° of motion vs 0.6° after fusion ^[47].
- It takes about 2–3 hours under general anesthesia. Many people go home the same day or after 1–2 nights ^[25, 73].

Artificial disc at C5–C6



Treated level keeps moving

Fusion (ACDF) at C5–C6



Treated level locked

Original schematic, motion exaggerated for visibility. Measured example: at 10 years in the Bryan trial, mean motion at the treated level was 8.69° with the artificial disc vs 0.60° after fusion ^[47]. Two-level Mobi-C patients kept a mean 10.1° (upper) and 8.3° (lower) of flexion-extension at 24 months ^[11]. Motion can be lost later if bone forms around the implant (heterotopic ossification) ^[44].

What recovery usually feels like

- A sore throat and some trouble swallowing for days to a couple of weeks are common ^[25, 77]. Hoarseness can happen and usually passes ^[27, 77].
- Pain at the back of the neck and between the shoulder blades is common ^[93, 94].
- Arm pain often improves first. Numbness is often the last thing to improve ^[94].
- Walking is encouraged right away ^[76, 27]. Many people are back to desk work within 1–2 weeks ^[25]. Full recovery can take up to 6 months ^[29].

How it compares with fusion

- Pooled trials found fewer repeat operations at the neighboring levels after disc replacement (4.3% vs 10.8% at 7 years for one level). The difference appeared around year 5 ^[52].
- Average symptom scores were slightly better, but the difference was small enough that patients may not notice it ^[54].
- Trouble swallowing was a bit less common after disc replacement (9.5% vs 12.1%) ^[58].

The main risks

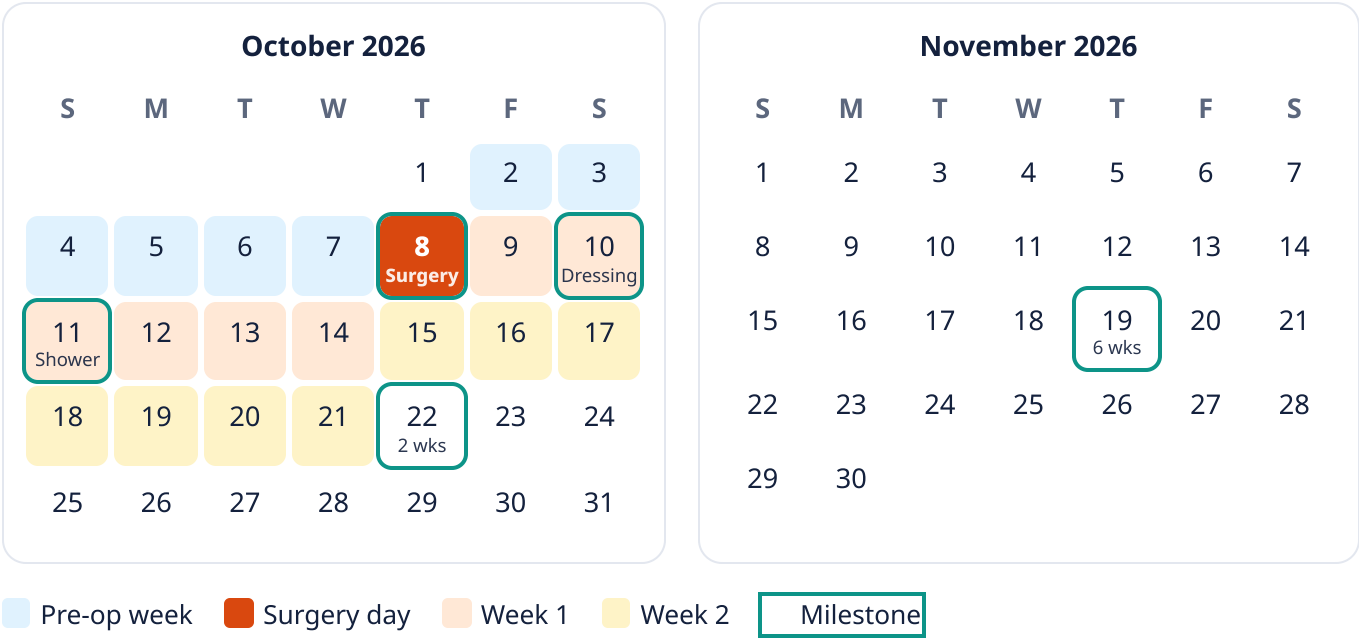
- Front-of-neck surgery risks include swallowing and voice problems, bleeding, infection, nerve injury and, rarely, breathing problems ^[27, 29].
- Over time, bone can form around the implant (heterotopic ossification), and some designs have had bone-loss (osteolysis) problems ^[61, 20]. See Risks.

Three things to remember

1. Any trouble breathing or fast-growing neck swelling means call 911 ^[92, 93].
2. Follow Dr. Rovner's team's instructions over anything general.
3. Walk, rest, eat soft foods, and ask for help.

Your Recovery Calendar

Your recovery, with real dates counted from surgery day (Thursday, October 8, 2026). These are typical timelines from published sources, so **follow your surgeon's instructions** if they differ. Everyone heals at their own pace. If you're reading this during recovery, today's section is highlighted.



Thursday, October 8, 2026

Surgery day

Surgery takes about 2–3 hours ^[25]. Dr. Rovner's practice says many of its patients go home the same day, and that cervical cases are outpatient or a one-night stay ^[73, 78]. Someone else drives you home ^[29, 87]. Stay inside the house for the first 24 hours ^[87].

Oct 8–10

The first nights

Sleep propped up in a recliner or on pillows ^[92]. A sore throat and trouble swallowing are common, so sip slowly and use ice chips or popsicles ^[92, 93]. Most people walk and eat on day 1 ^[27]. It helps to have someone with you at night at first ^[92]. Keep the red-flag card handy.

Oct 10–11

Days 2–3: dressing and first shower

The practice says to remove the dressing on day 2 or 3 and leave it off if the wound is dry. Shower once the wound is sealed (about 3 days) and dry, then blot dry and leave the steri-strips alone ^[76]. Everyday self-care like grooming and simple meals usually resumes around day 2 ^[29].

Oct 8–14

Week 1

Walk often; the practice discourages bed rest ^[76]. Avoid "excessive" lifting, bending and twisting ^[76]. No NSAIDs such as Advil or Aleve ^[76]. Watch for constipation from pain medicine ^[76]. Swallowing difficulty may last a week or two ^[77]. Use the daily log.

Oct 15–21

Week 2

Many people return to desk work, and some to driving, within 1–2 weeks ^[25, 71]. Don't drive while taking narcotic pain medicine, and ask the surgeon first ^[91, 93]. No baths, pools or hot tubs until the post-op visit ^[76]. First follow-up: To fill in: date of first post-op visit _____.

Oct 22 – Nov 4

Weeks 3–4

Light, regular activities usually resume between 2 and 3 weeks ^[29]. Swallowing solids and hoarseness can take a few weeks to settle ^[27]. Call if swallowing is getting worse rather than better ^[29].

Nov 5–18

Weeks 5–6

Light exercise at 4–6 weeks (expert consensus) ^[71]. Cleveland Clinic describes full activity except contact sports by 4–6 weeks ^[29].

Nov 19 – Dec 30

About 6 weeks (Thu, Nov 19) to 12 weeks

An expert panel agreed on more intense conditioning around 6 weeks ^[71]. Strenuous jobs usually take 6–12 weeks ^[25]. The practice says that because disc replacement doesn't depend on bone fusing, you can resume activities as tolerated ^[76]. Ask your surgeon what that means for you.

Around Jan 8, 2027

3 months

Selected athletes return to contact sport around 3 months after one-level surgery (surgeon consensus), with more caution after two levels ^[72]. Ask about anything high-impact.

Around Apr 8, 2027

6 months

Full recovery can take up to 6 months ^[29]. Pain and function keep improving over weeks to months, and disc replacement may not remove every bit of pain ^[25].

Around Oct 8, 2027

1 year and beyond

Follow-up visits check that the disc is working ^[29]. If your implant is an M6-C, the UK regulator advises yearly X-rays ^[20].

The evidence behind these timelines is in [Recovery week by week](#) (deeper reading).

Daily Recovery Log (Printable)

One row per day. Fill it in each evening; it takes two minutes and helps at your follow-up visit. The incision items match what MedlinePlus says to check daily: more redness or swelling, drainage, warmth, or the wound opening ^[88]. Anything on the red-flag card means call, so don't just log it.

Week 1

Days 0-6 · Oct 8-14							
DATE	PAIN (0-10)	MEDICINES & TIMES	WALKS (HOW MANY / MINUTES)	SWALLOWING	INCISION CHECK	TEMP (°F)	NOTES: SLEEP, BOWELS, QUESTIONS
Thu, Oct 8 Day 0	AM — PM —			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Fri, Oct 9 Day 1	AM — PM —			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Sat, Oct 10 Day 2	AM — PM —			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Sun, Oct 11 Day 3	AM — PM —			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Mon, Oct 12 Day 4	AM — PM —			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Tue, Oct 13 Day 5	AM — PM —			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Wed, Oct 14 Day 6	AM — PM —			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		

Week 2

Days 7-13							
DATE	PAIN (0-10)	MEDICINES & TIMES	WALKS (HOW MANY / MINUTES)	SWALLOWING	INCISION CHECK	TEMP (°F)	NOTES: SLEEP, BOWELS, QUESTIONS
Thu, Oct 15 Day 7	AM ____ PM ____			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Fri, Oct 16 Day 8	AM ____ PM ____			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Sat, Oct 17 Day 9	AM ____ PM ____			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Sun, Oct 18 Day 10	AM ____ PM ____			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Mon, Oct 19 Day 11	AM ____ PM ____			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Tue, Oct 20 Day 12	AM ____ PM ____			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		
Wed, Oct 21 Day 13	AM ____ PM ____			☐ easy ☐ OK ☐ hard	☐ dry ☐ red ☐ swollen ☐ draining		

Medicine schedule

MEDICINE	DOSE	HOW OFTEN	WHAT IT'S FOR	NOTES (E.G. CAUSES DROWSINESS OR CONSTIPATION)

The practice usually prescribes a narcotic pain medicine and sometimes a muscle relaxant, and says to avoid NSAIDs ^[76]. Many prescription pain pills already contain acetaminophen (Tylenol), so check before adding more ^[93].

Red-Flag Card: When to Call for Help

Print this card and keep it on the fridge. When in doubt, call. No one on the team minds a call. The categories below come from published instructions written for front-of-neck spine surgery. Your surgeon's instructions take priority.

Call 911 or go to the ER now

- **Any trouble breathing**, or a feeling that your breathing has changed. Swallowing trouble can be normal; breathing trouble never is ^[92, 93, 27].
- **Neck swelling that grows quickly or is large**, especially with trouble breathing ^[93, 94].
- **Chest pain** or sudden shortness of breath ^[93, 91, 88].
- **Significant new weakness**, or being unable to fully move your arms or legs ^[93, 94].
- **Loss of control of bowels or bladder** ^[94].

Call the surgeon's office right away

Office: (925) 275-0700 After hours: To fill in: after-hours number _____

- **Fever**: over 101°F that persists, per the practice ^[76]. MedlinePlus says chills or fever of 101°F or higher ^[88]. Ask the team for their exact number.
- **Swallowing that is getting worse** or not improving, trouble eating or drinking, or coughing when you swallow ^[29, 89].
- **Incision**: continuous bleeding; yellow or green drainage; more redness, swelling or warmth; or the edges opening ^[29, 88].
- **New numbness, tingling or weakness**, or a change from how you felt before surgery ^[29, 88, 94].
- **Severe pain** that your medicine doesn't control ^[29].
- **Calf pain or swelling**, which can be a sign of a blood clot ^[88, 91].
- Trouble passing urine, or a severe headache that won't go away with usual medicine ^[88, 94].

Usually expected (mention at your follow-up)

- A sore throat for several days, and some swallowing difficulty for days to weeks, especially with dry food or big pills ^[92, 27, 94].
- Hoarseness that slowly improves ^[27, 77].
- Aching at the back of the neck and between the shoulder blades ^[93, 94].
- Some bruising and swelling near the incision, and a little drainage at first ^[92, 76].
- A low fever in the first few days, which the practice describes as normal (see the note about their wording on Your care team) ^[76].

Hospital and practice handouts list slightly different symptoms and thresholds. This card combines them. A doctor would treat breathing trouble and fast-growing neck swelling as the most urgent.

For Wesley: How to Help

Thank you for being Sarah's person through this. Your most important jobs are to keep her safe, keep her comfortable, and notice changes early. The rest is snacks and patience.

The first 72 hours

- **Drive her home and stay.** A responsible adult has to take her home, and she should stay in the house for the first 24 hours ^[87]. One spine manual suggests having someone there at night for the first night or so ^[92].
- **Watch her breathing and her neck.** Any breathing trouble, or neck swelling that grows quickly, means call 911 ^[92, 93]. Some bruising and mild swelling is expected ^[92].
- **Medicines on schedule.** Write each dose in the daily log. The practice says narcotics cause constipation, so use the minimum, and no NSAIDs ^[76]. Check whether her prescription already contains acetaminophen before giving Tylenol ^[93].
- **Help her walk.** Short, frequent walks around the house. The practice discourages bed rest ^[76]. Hold the stair rail and stay with her on stairs ^[93].
- **Throat comfort.** Ice chips and popsicles help a sore throat ^[93]. Warm or cold drinks are both fine ^[92].
- **Dressing and shower.** The practice says the dressing comes off on day 2–3 and she can shower once the wound is sealed (about 3 days) and dry. Be there for the first shower ^[76, 88].

The first two weeks

- Do the lifting and reaching: groceries, laundry, vacuuming, high or low shelves ^[91]. Norton advises nothing over 10 lb until the surgeon says it's OK ^[93]. Dr. Rovner's practice says avoid "excessive" lifting, bending and twisting ^[76].
- **Driving her:** she shouldn't drive while on narcotic pain medicine or before she can turn to check her blind spot ^[91]. Ask the surgeon before she drives. Keep car trips short at first, and stop to stretch on longer rides ^[88].
- Keep an eye on the incision each day ^[88] and on how her swallowing is going. It should slowly get easier ^[29].
- Make sure she gets to the first follow-up: To fill in: date of first follow-up _____.

Meals and soft foods

The practice says there are normally no diet restrictions, and that this is not a time to diet. She needs calories to heal ^[76]. For a sore throat or swallowing discomfort:

- **Soft, moist foods:** scrambled eggs, mac and cheese, soups and stews, mashed potatoes with gravy, cooked cereal, cooked vegetables, cottage cheese, custard, pudding ^[90]. Add broth, gravy or sauce to moisten food ^[90].
- **How to eat:** sit upright; small bites of less than a teaspoon; chew well and swallow before the next bite; don't talk while swallowing; stay upright for 30–45 minutes after eating ^[89]. Take small bites and add regular foods back as she feels better ^[93].
- **Drinks:** thin liquids can be harder at first, so drink slowly. Thicker drinks like smoothies may be easier ^[92].
- **Where advice differs:** one practice suggests avoiding dairy until swallowing feels normal, because it can cause mucus ^[92]. The VA list includes dairy, such as cottage cheese and custard ^[90]. The VA suggests a sip after each bite, while MedlinePlus says not to wash solids down with liquids ^[90, 89]. Go with whatever feels easiest for her, and ask the team if swallowing is hard.
- Red meat and iron help with post-op anemia, according to the practice ^[76]. Fluids and fiber help with constipation ^[91, 93].

Emotional support

- Feeling anxious before surgery is normal ^[92]. Recovery has good days and slow days.
- Accept help from friends and family. Patient booklets suggest a list of jobs they can take on: meals, errands, rides, laundry, yard work ^[91]. One manual says a support person helps patients have a smoother recovery ^[92]. Point them to the [Meals & Visits sign-up](#) and post news on [Recovery updates](#).
- Practical tips (not from a medical source): keep visits short, protect her rest, celebrate small wins like the first shower or the first longer walk, and take breaks for yourself too.

What to watch for

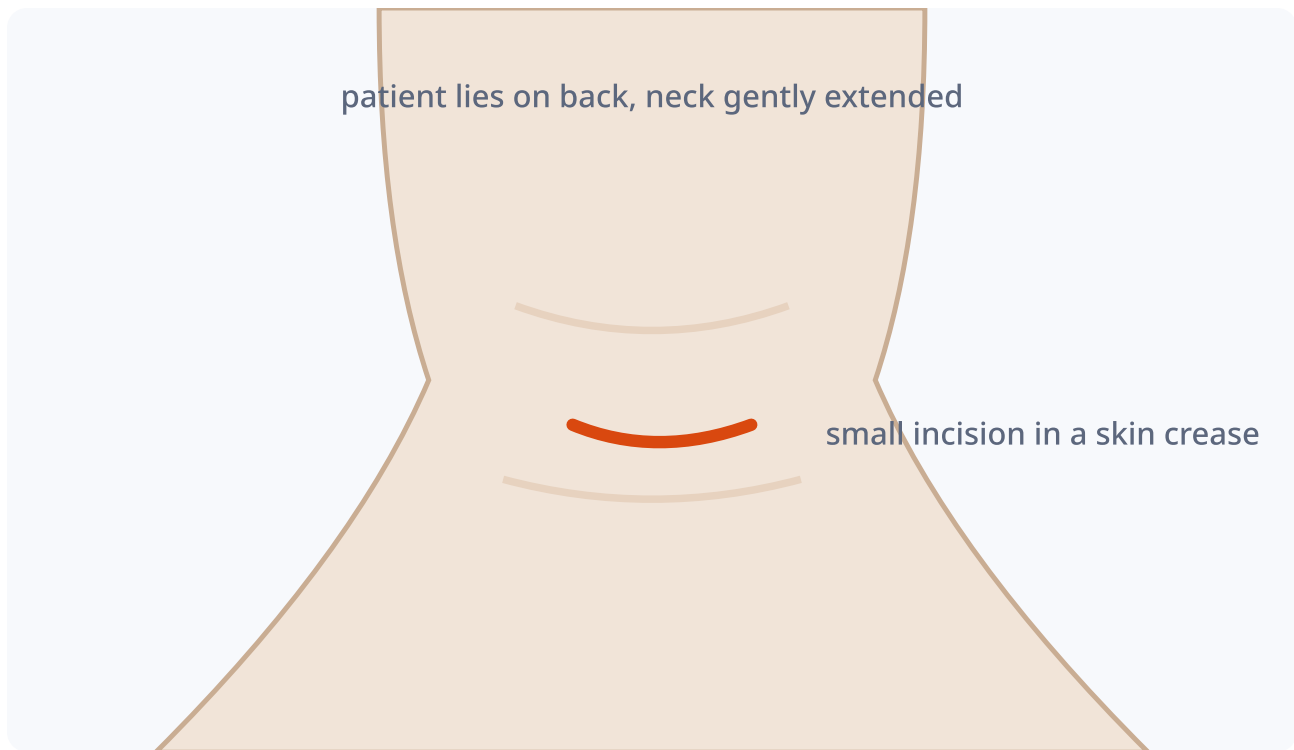
Keep the red-flag card on the fridge. Breathing trouble or fast-growing neck swelling means call 911. Fever, worsening swallowing, changes in the incision, or new weakness or numbness means call the surgeon ^[92, 93, 29].

KNOW

The Operation, Step by Step

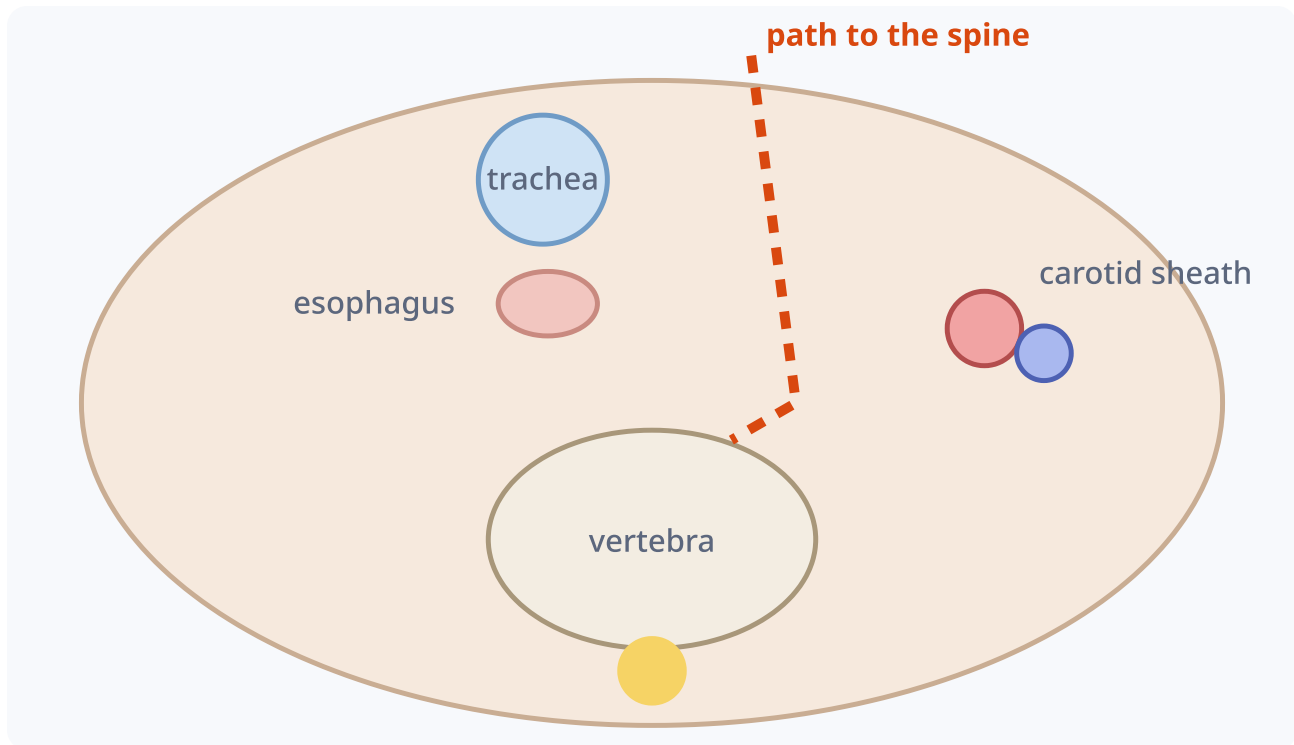
Cervical disc replacement is done through the front of the neck, the same approach as fusion (ACDF). What differs is the final step: a moving implant goes into the disc space instead of a bone graft or cage ^[27].

1. Positioning and incision



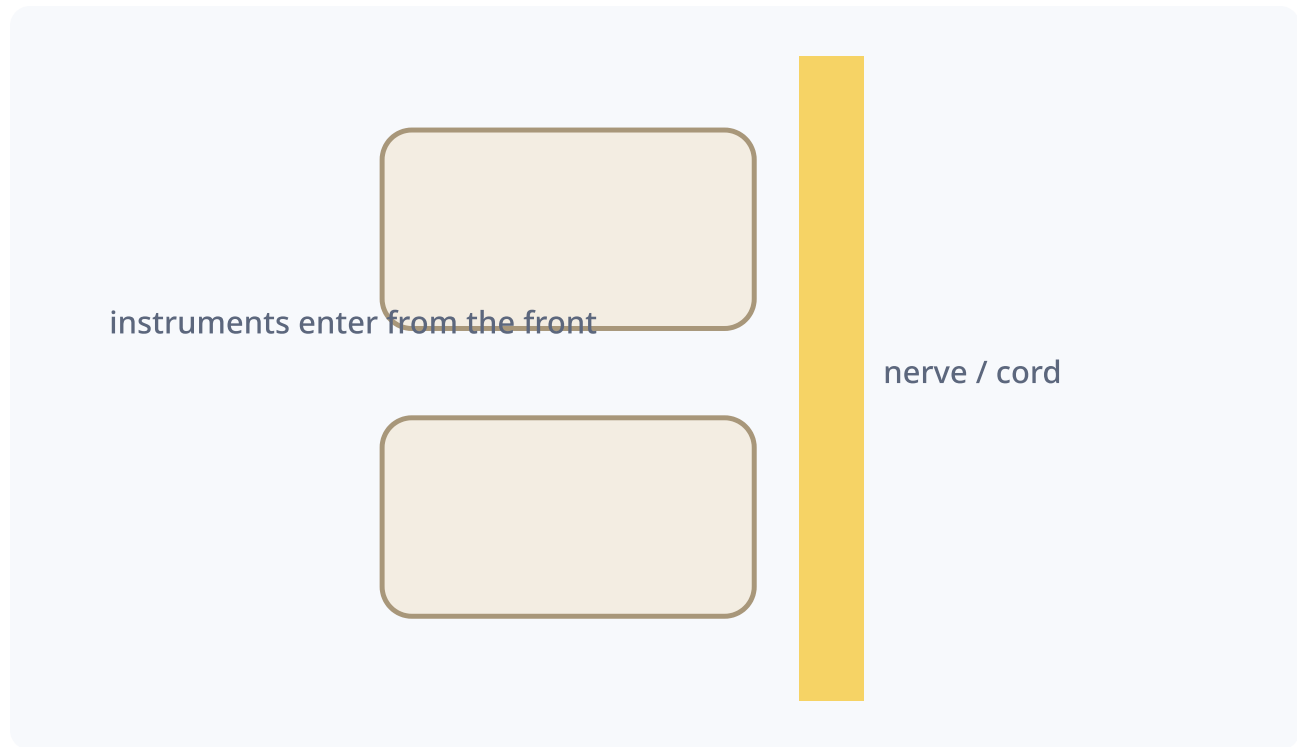
Under general anesthesia, lying on your back, a short incision is made on the front of the neck, usually in a skin crease ^[25, 29]. AAOS describes anterior incisions of 1 to 4 inches ^[27].

2. Anterior approach



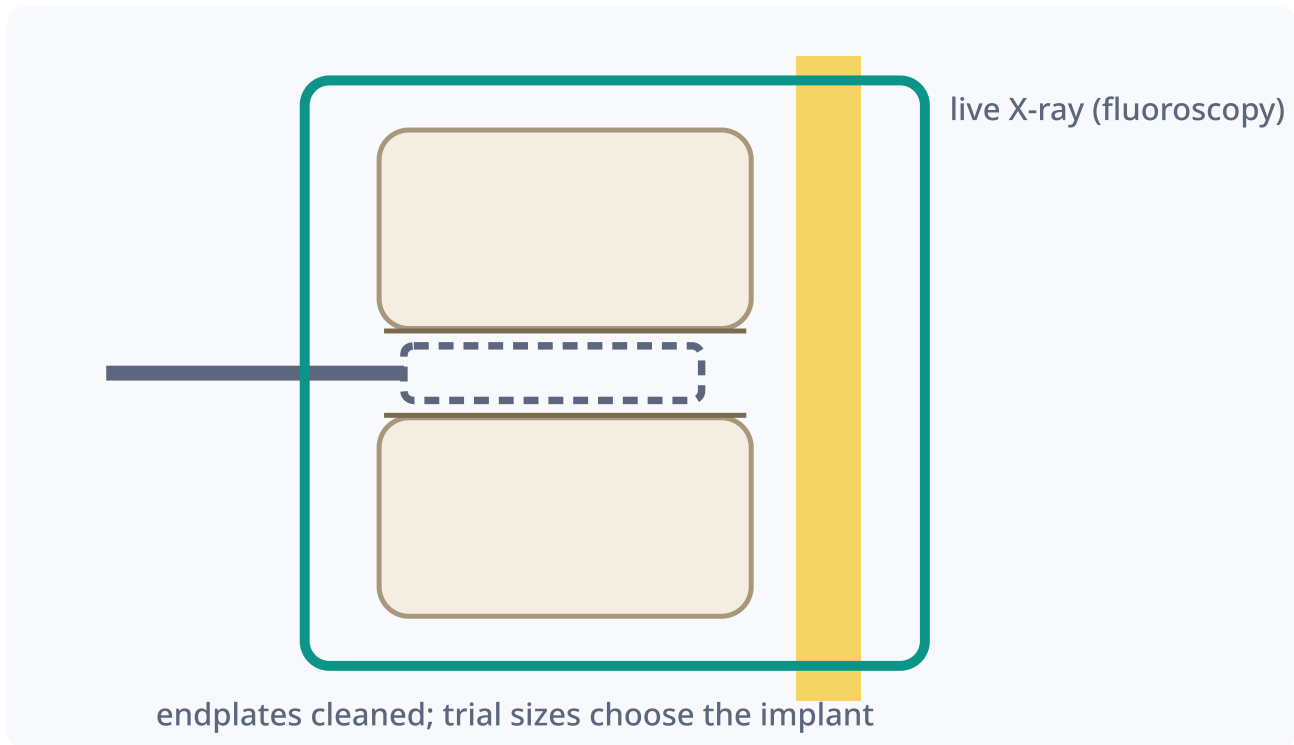
The surgeon works between natural tissue layers, moving the windpipe (trachea) and esophagus to the side and protecting the carotid artery, to reach the front of the spine ^[29, 25].

3. Discectomy and decompression



The damaged disc is removed, along with disc fragments or bone spurs that press on the nerve root or spinal cord (decompression) ^[27, 22].

4. Endplate preparation, sizing and X-ray check



The bony endplates are prepared and trial implants are used to pick the size; live X-ray (fluoroscopy) checks position throughout ^[29, 25, 22].

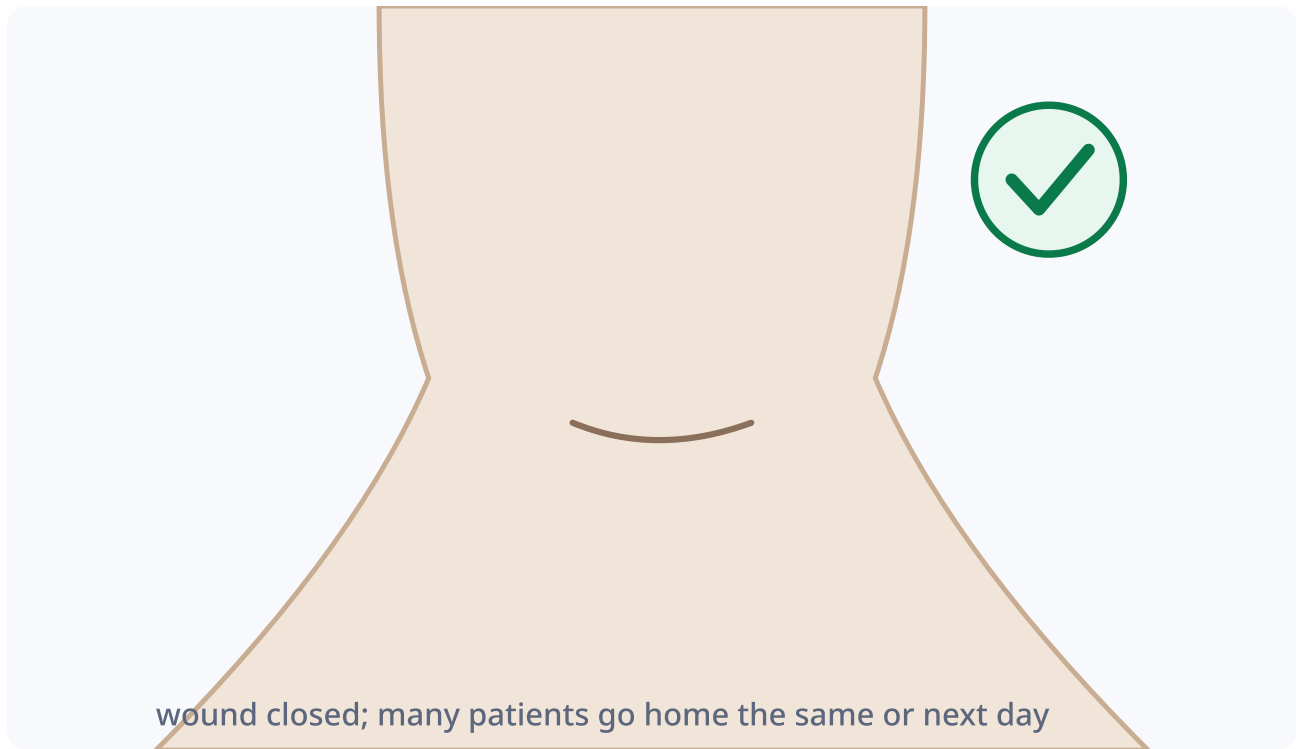
5. Implant insertion



implant designs differ (materials, core, fixation)

The artificial disc is inserted into the cleaned disc space. Designs differ in materials and how they grip bone (keels, spikes or coatings) ^[25, 18, 5].

6. Closure and wake-up



Tissues fall back into place and the incision is closed ^[29]. Surgery usually takes 2 to 3 hours; many people stay 1 to 2 days, some go home the same day ^[25].

Original simplified schematic of a typical anterior cervical disc replacement. Details vary by surgeon and implant. In print, every step is shown as a still image.

Before surgery

- **Imaging and tests:** X-ray, MRI or myelogram; sometimes CT and bone-density testing ^[29, 25, 22].
- **Medications:** your surgeon will review medicines, vitamins and supplements, and some may need to be stopped. Don't stop any medicine unless told to ^[25, 29].
- **Nicotine:** you may be asked to stop tobacco or nicotine products at least 4 weeks before surgery ^[29].
- **Fasting:** usually nothing to eat or drink after the night before surgery ^[25].
- **Ride home:** someone else must drive you home ^[29].

Step by step

1. **Anesthesia and positioning.** General anesthesia through an IV, lying on your back ^[25, 29].

2. **Incision.** Made on the front of the neck, usually in a skin crease. Its exact location and length vary [25, 29].
3. **Anterior approach.** The trachea, esophagus and blood vessels are moved aside to reach the front of the spine [29, 25].
4. **Imaging.** Fluoroscopy (live X-ray) is used throughout to confirm the level and position [29, 25].
5. **Discectomy and decompression.** The damaged disc is removed, along with bone spurs or fragments pressing on the nerve root or cord. CPT code 22856 explicitly includes "end plate preparation" and "osteophylectomy for nerve root or spinal cord decompression and microdissection" [22].
6. **Implant insertion.** The artificial disc is sized and inserted, and its position is checked on imaging [25].
7. **Closure.** Structures are moved back into place, the incision is closed and dressed [29, 25].

How long does it take?

- AAOS: most disc replacements take 2 to 3 hours [25].
- Two-level Mobi-C trial: mean operative time 2.1 hours (ADR) vs 1.8 hours (ACDF) [11].

Hospital stay

- Usually 1–2 days; some people go home the same day [25].
- In the two-level Simplify study, mean stay was 0.59 days vs 1.31 days for historical ACDF controls. FDA's summary says this probably reflects more outpatient surgery sites and is not clinically meaningful [18].
- In the 2013 two-level Mobi-C trial, mean stay was 2.2 vs 2.4 days [11].

Right after surgery

- You will be monitored in the recovery room as you wake up [29].
- Temporary difficulty swallowing for a few days is common [25]. After anterior neck surgery, AAOS notes swallowing solids may be hard for a few weeks and hoarseness can occur [27].
- Most patients are able to walk and eat on the first day after surgery [27].
- Your surgeon may have you wear a protective collar at first, and excessive neck motion is limited while bone grows onto the implant [25].

Continue to the week-by-week recovery timeline.

Recovery Week by Week: Work, Driving and Sports

Recovery after disc replacement is usually faster than after fusion because there is no fusion to wait for. Timelines still vary by person, by job, and by surgeon. Scroll to animate the timeline. Each step shows where the guidance comes from, and sources disagree slightly.

DAY OF SURGERY – DAY 1

Hospital or same-day discharge

Most people stay 1–2 days; some go home the same day ^[25]. Walking and eating usually start on the first day ^[27]. Someone else must drive you home ^[29].

DAY 2

Daily self-care

Activities of daily living such as grooming and preparing meals usually resume by the second day ^[29]. Swallowing may feel difficult for a few days ^[25].

WEEKS 1–2

Desk work, driving, travel

AAOS: most patients return to driving and desk jobs within 1–2 weeks ^[25]. An expert panel agreed patients may return to walking, social activities, sedentary work, air travel and sexual activity within 2 weeks ^[71]. Your surgeon decides when driving is safe for you.

WEEKS 2–3

Regular light activities

Cleveland Clinic: light activities between 2 and 3 weeks ^[29]. Avoid heavy lifting and aerobic exercise until cleared ^[29].

WEEKS 4–6

Light exercise, physical therapy, most activities

Light physical activity at 4–6 weeks (expert consensus) ^[71]. Full activities except contact sports by 4–6 weeks ^[29]. Range-of-motion and strengthening exercises often begin around 4–6 weeks after anterior neck surgery, sometimes with PT ^[27].

ABOUT 6 WEEKS

Training and heavier work

The expert panel agreed on intense conditioning and sport-specific training at 6 weeks, but had only weaker agreement on physically demanding work at 4–6 weeks and high-intensity sport at 6 weeks ^[71]. In the Bryan trial, 49.2% of disc-replacement patients were back at work at 6 weeks vs 39.4% after fusion ^[49].

WEEKS 6–12

Strenuous jobs

AAOS: returning to strenuous jobs usually takes 6–12 weeks ^[25]. Mean return to work in the two-level Mobi-C trial was 45.9 days vs 66.8 days after fusion ^[11].

ABOUT 3 MONTHS

Contact and collision sports (selected athletes)

In a 34-surgeon consensus, the most endorsed timelines after one-level replacement were 6 weeks or 3 months for practice and 3 months for games. Only 23.5% would let an athlete return to high-impact, high-frequency sport after a two-level replacement, compared with 57.6% after one level ^[72].

UP TO 6 MONTHS

Full recovery

It may take up to 6 months to recover fully ^[29]. Improvements in pain and disability come over weeks to months, and disc replacement improves but does not completely eliminate pain ^[25].



1 YEAR AND BEYOND

Follow-up imaging

Follow-up visits check that the disc is working ^[29]. Work outcomes: disc-replacement patients had higher odds of working at 6 weeks, 3 months and 1 year, but not at 2 years or later ^[57]. People with an M6-C should have annual X-rays according to the UK MHRA ^[20].

Return to work: the evidence

- Meta-analysis of 16 studies (5657 patients): higher odds of working at 6 weeks (OR 1.33), 3 months (OR 1.58) and 1 year (OR 1.35); about 9.9 days earlier on average; no difference at 2 years or later ^[57].
- Bryan trial: more patients back at 6 weeks, no difference by 6 months ^[49].

Driving

AAOS says most people can drive within 1–2 weeks ^[25]. Ask your surgeon when you may drive, especially if you are still taking prescription pain medicine or wearing a collar.

Sports

- General patients: full activity except contact sports by 4–6 weeks ^[29]; sport-specific training at 6 weeks (expert consensus) ^[71].
- High-impact athletes: much less agreement, and more caution after two-level surgery ^[72].

Where recovery advice differs

- Light activities: "1–2 weeks" for desk work and driving (AAOS) vs "2–3 weeks" for regular light activities (Cleveland Clinic) ^[25, 29].
- Full recovery: "full activities excluding contact sports by 4–6 weeks" vs "up to six months" for full recovery, both from Cleveland Clinic. These describe different milestones ^[29].
- Strenuous work: 6–12 weeks (AAOS) vs only weak expert consensus for 4–6 weeks ^[25, 71].

Risks and Complications

Disc replacement shares the general risks of any neck surgery done from the front, and adds implant-specific risks such as heterotopic ossification, wear and osteolysis. The rates below come from specific studies and vary widely with device, follow-up length and how carefully problems were looked for.

Risks of any anterior cervical surgery

- **Any cervical spine surgery:** infection, bleeding, nerve injury, spinal cord injury, reaction to anesthesia, a dural tear (tear of the sac around the nerves), failure to relieve symptoms, needing more surgery later, and rarely life-threatening problems such as heart attack, lung complications or stroke ^[27].
- **Front-of-neck (anterior) approach:** soreness or difficulty swallowing, voice changes, breathing difficulty, injury to the esophagus, and misplaced, broken or loosened implants ^[27]. Cleveland Clinic adds a sore throat that comes and goes ^[29].
- **Disc-replacement-specific, long term:** adjacent segment degeneration, artificial-disc-related failures, heterotopic ossification, and possibly a future operation ^[29].

Reported complication rates (selected)		
COMPLICATION	REPORTED RATE	SOURCE / CONTEXT
Dysphagia (swallowing difficulty)	9.46% ADR vs 12.09% ACDF (RR 0.76)	Meta-analysis of 10 RCTs, 2711 patients [58]
Dysphagia, wide range	Reported as high as ~70%	Narrative review; depends on definition [66]
Dysphagia/dysphonia, two-level	16.7% Mobi-C vs 22.9% ACDF through 24 months	Two-level IDE trial [11]
Laryngeal nerve injury (hoarseness)	~0–1.25%	Narrative review [66]
Horner syndrome; hematoma; device extrusion	~0.06%; ~0.01%; ~0.3%	Narrative review [66]
Short-term complications overall	0.8% (vascular) to 4.7% (dysphagia)	Meta-analysis of 53 studies, 3223 patients [63]
Any complication, matched national data	8.77% ADR vs 12.20% ACDF	PearlDiver insurance-claims database [60]

Heterotopic ossification (HO)

HO is bone forming in soft tissue around the implant. Advanced HO can bridge the level and stop it moving, effectively a spontaneous fusion [61, 62].

- Pooled prevalence: 44.6% at 12 months and 58.2% at 24 months; advanced (grade III–IV) HO 11.1% and 16.7% [61].
- Prestige LP two-level, 10 years: grade IV HO 8.2% (upper level) and 10.3% (lower level). Grade III–IV was 42.4% at 7 years and 39.0% at 10 years [44].
- Two-level Mobi-C at 24 months: clinically relevant HO (grade III/IV) at 11.4% of upper and 10.1% of lower levels [11].
- Range across studies: ~7.3% to 69.2% [66].
- **Does it matter?** Having any HO was not linked to worse pain or disability scores, but high-grade HO was linked to worse pain [62]. HO was associated with adjacent-segment degeneration and, perhaps surprisingly, with less dysphagia [63].

- **Risk factor:** exposed endplate (an implant footprint that doesn't cover the bone) was a significant risk factor for HO ^[64].

Osteolysis (bone loss around the implant)

- A systematic review found asymptomatic osteolysis in 8–64% of patients (44–64% excluding one outlier study). Severe bone loss exposing the implant occurred in under 4%. Most cases were mild and did not need revision ^[65].
- Symptomatic osteolysis often required implant removal and conversion to fusion ^[65].
- Device-specific signals: M6-C (34% revised at a single center; UK alert recommends annual X-rays) and Simplify (MAUDE reports of PEEK wear and osteolysis). See [safety alerts](#) ^[67, 20, 69, 70].

Repeat surgery

- Index-level reoperation by 7 years: 5.2% ADR vs 12.7% ACDF (one level), 4.2% vs 13.5% (two levels) ^[52].
- Implant failure ~3.3–3.7% (review) ^[66].
- Revising or removing a cervical artificial disc has its own billing codes (CPT 22861 and 22864) ^[22].

Adjacent segment degeneration and disease

Losing motion at a fused level may speed up wear at the neighboring levels. Keeping motion with disc replacement is meant to prevent this ^[54, 52]. Radiographic adjacent-segment degeneration is common after disc replacement too: it was 36.0% pooled at long-term follow-up in one meta-analysis ^[63]. What matters most to patients is whether another operation is needed. Those rates are lower after disc replacement in pooled trials, with the difference showing at around 5 years ^[52, 53].

When to call your surgeon

Cleveland Clinic advises contacting your provider for continuous bleeding from the incision, yellow or green drainage, severe pain, fever, worsening or persistent difficulty swallowing or eating, or new weakness or numbness ^[29]. Breathing difficulty is a recognized risk of anterior neck surgery ^[27]. Ask your surgeon which symptoms need emergency care.

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How to read these sources: PubMed abstracts summarize studies; FDA "SSED" documents and device labels are regulatory summaries; insurer policies (Aetna, Medicare LCD) describe coverage, not medical truth; manufacturer documents are marked *Industry* and should be read with that in mind. Most pivotal disc-replacement trials were sponsored by the device makers.

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Not medical advice. Educational summary of published research, FDA records, insurer policies and professional-society patient pages. Statistics come from specific studies with specific patients and may not apply to you. Device availability, labeling and insurance policies change; confirm current details with your surgeon and insurer.

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